

90-DAY CLINICIAN EXPORT

Allergy visit summary

Sample Patient
DOB: 04/18/1998

May 1 - July 31, 2026
Prepared Aug 4, 2026

ILLUSTRATIVE SAMPLE

This document demonstrates an Immunity export using fictional data. Detected patterns are associations for clinician review, not confirmed allergies or medical diagnoses.

42

food entries

5

symptom episodes

3

repeated-pattern events

0

emergency visits

Primary pattern for review

Possible cashew / pistachio association

Three similar reactions followed foods that contained, or may have contained, cashew or pistachio.

â Symptoms began 12-38 minutes after eating.

□

â Mouth itching and hives appeared in 3 of 3 events.

□

â Two restaurant entries have incomplete ingredient details.

□

PATTERN STRENGTH

Moderate

Repeated exposure, timing, and symptoms support review. Incomplete labels and one tolerated precautionary-label exposure limit certainty.

Safety and symptom snapshot

No breathing or cardiovascular symptoms logged

No wheezing, throat tightness, fainting, or low blood-pressure symptoms were recorded.

Medication availability often missing

Whether epinephrine was available was not recorded in 4 of 5 symptom episodes.

Clinical context

History in app: intermittent eczema and exercise-related asthma. No confirmed tree-nut allergy is recorded. Current prescribed medications and emergency action plan should be reconciled during the visit.

PATTERN EVIDENCE

What Immunity connected

FINDING	OBSERVED EVIDENCE	INTERPRETATION LIMIT
Possible shared trigger	Cashew or pistachio appeared, or may have appeared, in 3 similar episodes.	Two restaurant ingredient lists are incomplete.
Consistent timing	Symptoms began 12-38 minutes after eating; median onset was 18 minutes.	Times were self-reported and may be approximate.
Repeated symptoms	Mouth itching and hives were logged in all 3 pattern events.	Photos were available for only 2 events.
Possible cofactor	Exercise occurred within 2 hours before 2 events.	The dataset is too small to separate exposure from cofactor effects.
Conflicting evidence	One item labeled 'may contain tree nuts' was tolerated.	A precautionary label does not confirm actual allergen presence.

Reaction timeline

DATE	EXPOSURE	ONSET	SYMPTOMS	RESPONSE / OUTCOME
May 14	Pistachio gelato about 1/2 cup	18 min	Mouth itching; facial hives	Cetirizine; resolved in 75 min
Jun 3	Restaurant pesto pasta about 1 serving	38 min	Hives; stomach discomfort	Cetirizine; resolved in 2 hr
Jul 19	Vegan cheese spread about 2 tbsp	12 min	Mouth itching; hives	Cetirizine; resolved in 60 min

Tolerated exposures

6x
Peanut
without logged reaction

4x
Almond
without logged reaction

11x
Milk
without logged reaction

8x
Egg
without logged reaction

3x
Sesame
without logged reaction

Tolerated exposures appear beside suspected triggers because negative evidence helps a clinician judge consistency. They do not establish that a food is safe for unsupervised reintroduction.

Other activity during this period

â

Two isolated congestion episodes occurred on days the patient marked as high pollen.

â

One eczema flare lasted four days; no repeated food association was found.

â

No nighttime reactions, emergency treatment, or epinephrine use were logged.

VISIT PREPARATION

Treatment, gaps, and next questions

Medication and response

MEDICATION	LOGGED USE	OBSERVED ENTRY
Cetirizine	5 doses	Symptoms improved after 45-90 minutes in 3 reaction entries.
Albuterol	1 use	Used during exercise; no allergic reaction was logged with that use.
Epinephrine	0 uses	Availability was not recorded in 4 of 5 symptom episodes.

Data completeness

Symptom onset	100%	Exact amount	60%
Symptom resolution	80%	Ingredient label or photo	60%
Context and cofactors	80%	Emergency medication available	20%

Questions highlighted for the allergist

1. Does this history support evaluating cashew, pistachio, or another shared ingredient?
2. Could exercise have contributed to the repeated pattern?
3. How should the patient distinguish a mild reaction from one requiring epinephrine?
4. Which foods should remain in the diet, and which - if any - should be avoided pending evaluation?
5. What additional details would be most useful during any future episode?

Clinician notes

Assessment

Testing / evaluation plan

Updated action plan and follow-up

Method note

Immunity groups repeated combinations of exposures, timing, symptoms, context, treatment, and outcomes. Pattern strength reflects repetition and record completeness; it is not the probability of an allergy and does not predict the severity of a future reaction. Clinicians should reconcile this export with the medical record and their independent assessment.

Clinical framing references: American College of Allergy, Asthma & Immunology, Food Allergy Testing and Diagnosis; American Academy of Allergy, Asthma & Immunology, Food Allergy and Anaphylaxis patient resources. Accessed August 4, 2026.